

EPIDERMOLYSIS BULLOSA

AT A GLANCE

Family of inherited genodermatoses characterized by blistering in response to minor trauma.

- Blistering level categorizes EB into simplex, junctional, and dystrophic subtypes.
- Cutaneous involvement ranges from localized to widespread, depending on subtype.
- Extra-cutaneous manifestations may affect oropharynx, trachea, esophagus, eyes, teeth, nails, and hair—severity varies by subtype.
- Diagnosis is established by immunofluorescence and/or electron microscopy, followed by genetic (DNA) analysis.

Epidermolysis Bullosa Simplex (EBS)

EBS is characterized by intraepidermal blistering due to mutations in keratin genes (KRT5 or KRT14). Clinical severity varies from mild to severe across subtypes. Most common forms are autosomal dominant and include:

- Generalized (Koebner)
- Localized (Weber-Cockayne)
- Herpetiform (Dowling-Meara)

Rare variants include EBS-Ogna, EBS superficialis, and EBS with mottled pigmentation.

Generalized Epidermolysis Bullosa Simplex (Koebner Variant)

- Onset at birth or early infancy.
- Generalized blistering, most prominent on hands, feet, and extremities.
- Blisters heal with post-inflammatory hyper- or hypopigmentation.

- Atrophy and milia are rare and less common than in other EB subtypes.
- Palmar-plantar hyperkeratosis and erosions may occur.
- Thickening of soles often develops in later childhood.
- Mild oral mucosal involvement possible, but typically improves with age.
- No severe growth retardation.

Epidermolysis Bullosa Herpetiformis (Dowling-Meara)

- Presents at birth with generalized blistering.
- Most severe form of EBS.
- Characterized by grouped, "herpetiform" blisters that may appear spontaneously.
- Oral mucosa frequently involved, sometimes with extensive erosions.
- Milia may develop in infancy but often resolve during childhood.
- Caused by mutations in *KRT5* or *KRT14*, with more pronounced keratin clumping on histology compared to other EBS subtypes.